

Community Pharmacy

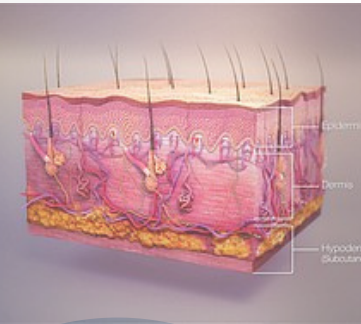
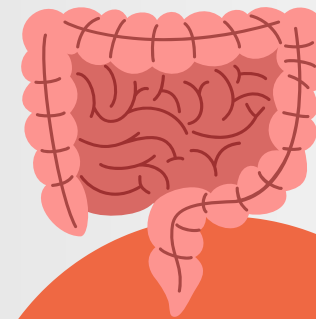
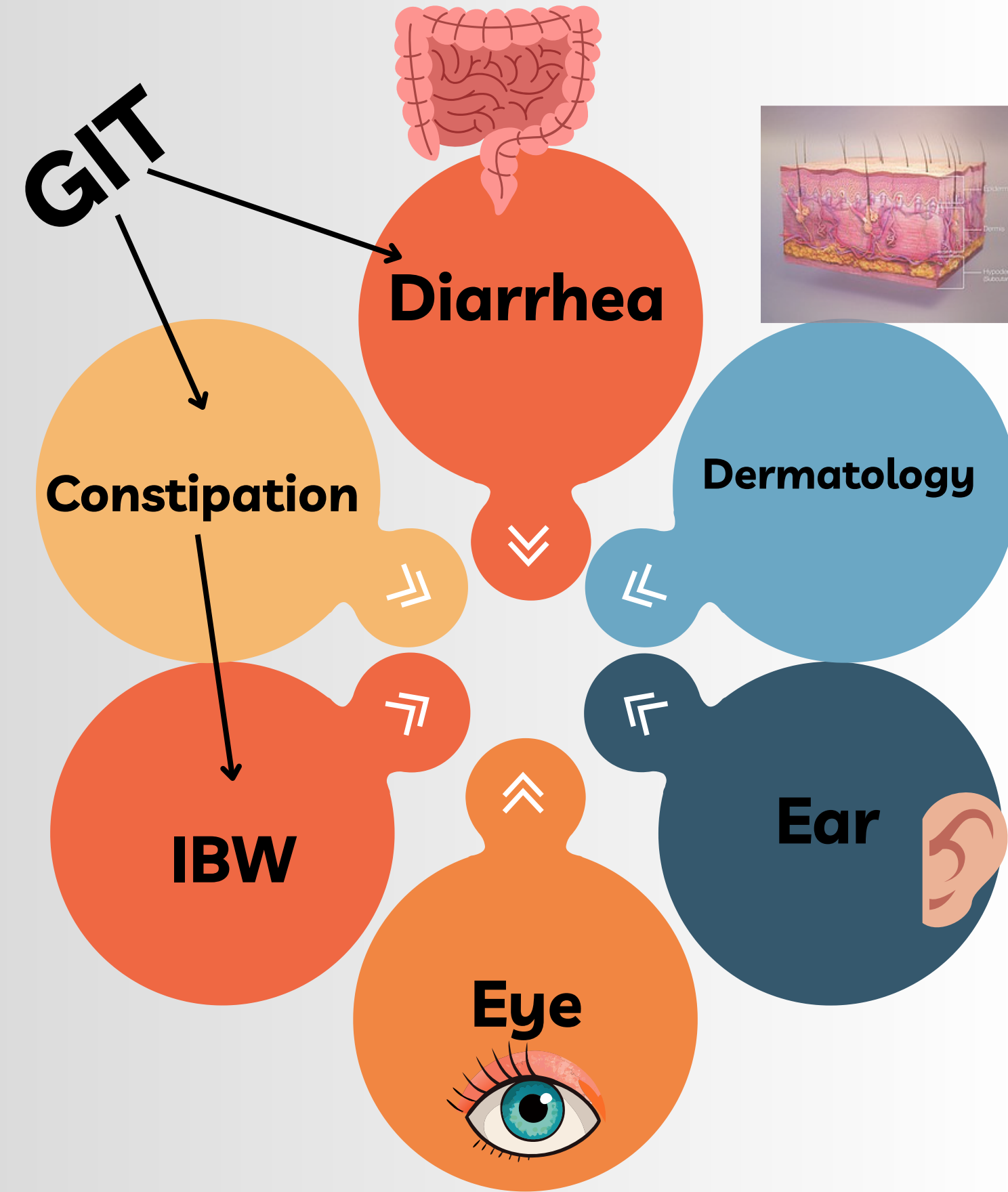
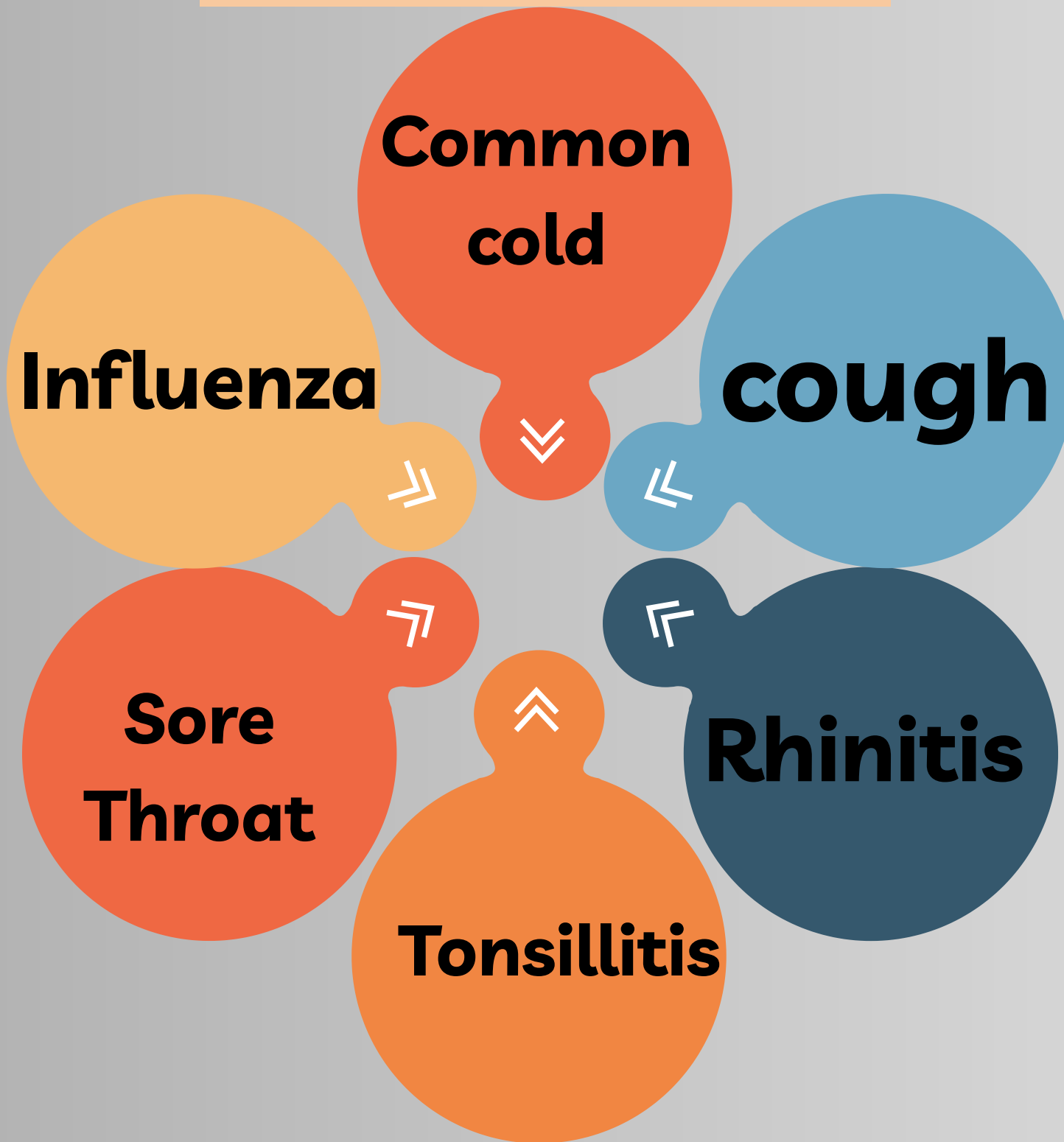
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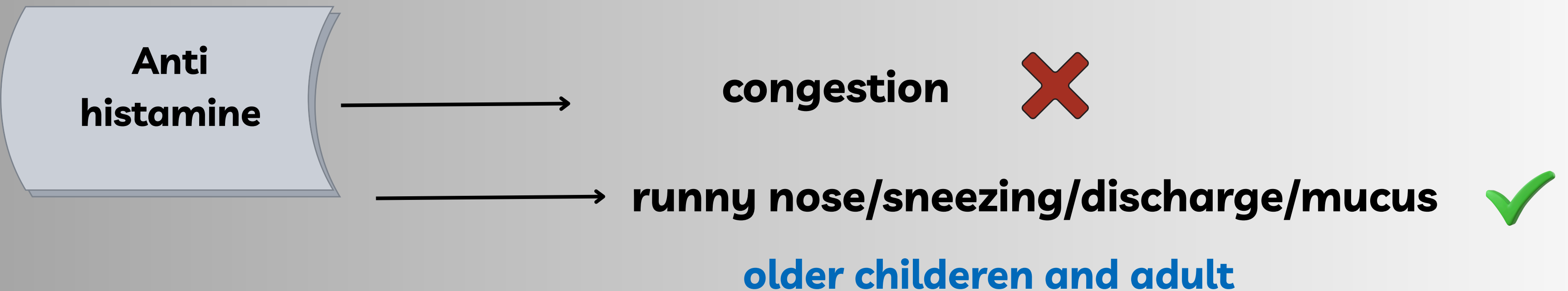
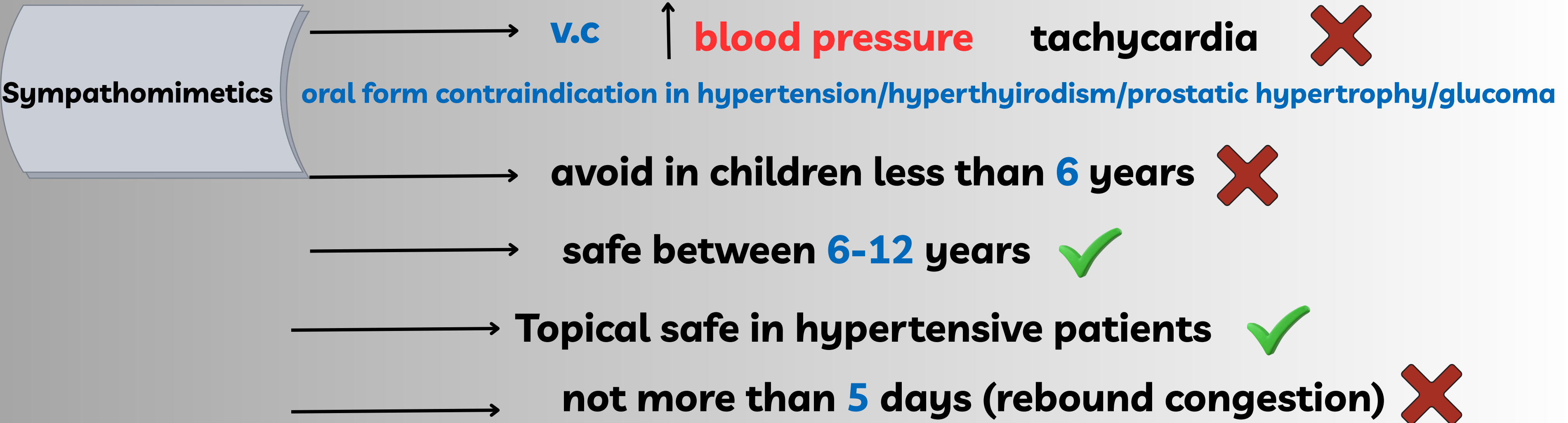
Dr/Merna aboelnour

Respiratory



	Common cold	Influenza
ONSET	More gradual	Sudden
FEVER	Low Un common	High ✓
stuffy nose Sneezing Sore throat	Common	Occasional
Cough chest pain	Mild to moderete	Severe Common dry ✓
Headache	Mild Uncommon	Severe Common ✓
Myalgia Arthralgia	Uncommon	Severe Common ✓

	Common cold	Influenza
Anti viral	✗	✓ Amantadine Zanamivir/Oseltamivir
Sympathomimetics Decongestant	Oxymetazoline pseudoephedrine phenylephrine	Oxymetazoline pseudoephedrine phenylephrine
Anti histamine	Diphenhydramine (sedation) loratidine (non-sedation)	Diphenhydramine (sedation) loratidine (non-sedation)



Sore Throat



Definition

1

Pain in any part of throat mucosa due to common cold (pharynx → pharyngitis; tonsils → tonsillitis).

Distinction

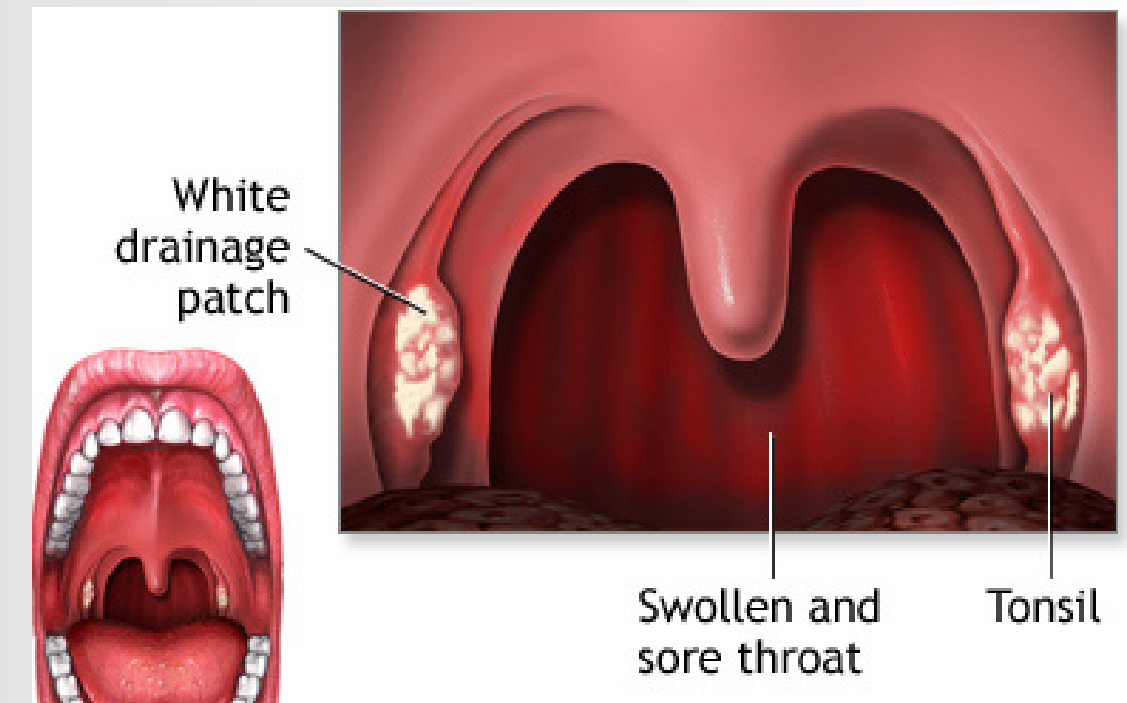
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Between pharyngitis and tonsillitis is unclear.

Age

3

<3 yrs → strep rare
5–10 yrs + 15–25 yrs → strep common; Children →; Adolescents → glandular fever;
Older age → carcinoma risk.



Cause

● **Viral 70–90% of cases**

● **Bacterial ~10–30% (most common:
Group A beta-hemolytic
Streptococcus / *S. pyogenes*)**

1 cough

2 pharyngeal inflammation **ulcer** ±
exudate.

3 fever, headache

1 **absence** of cough

2 Tonsillar exudate

3 fever, **tender** cervical lymph nodes

Complications of Streptococcal Throat

- 
- Otitis media
 - Acute sinusitis

Local anesthetics

Benzocaine

Lidocaine

**Antibacterials
/Antiviral**

**Penicillin or macrolides (if
allergy)**

**Anti-
inflammatories**

Benzydamine spray

Analgesics

Flurbiprofen

Paracetamol,

ibuprofen effective

Aspirin/saltwater gargles

Benzydamine spray

Rinse

Lidocaine

Flurbiprofen

>12 years

Benzocaine

**Lozenge >3 yrs;
Spray >6 yrs**

Benzydamine spray

**Spray/lozenge
>6 yrs**

Pregnancy

Flurbiprofen



avoid in 3trimester) pregnancy

Lidocaine



Not safe

Benzydamine spray
Rinse



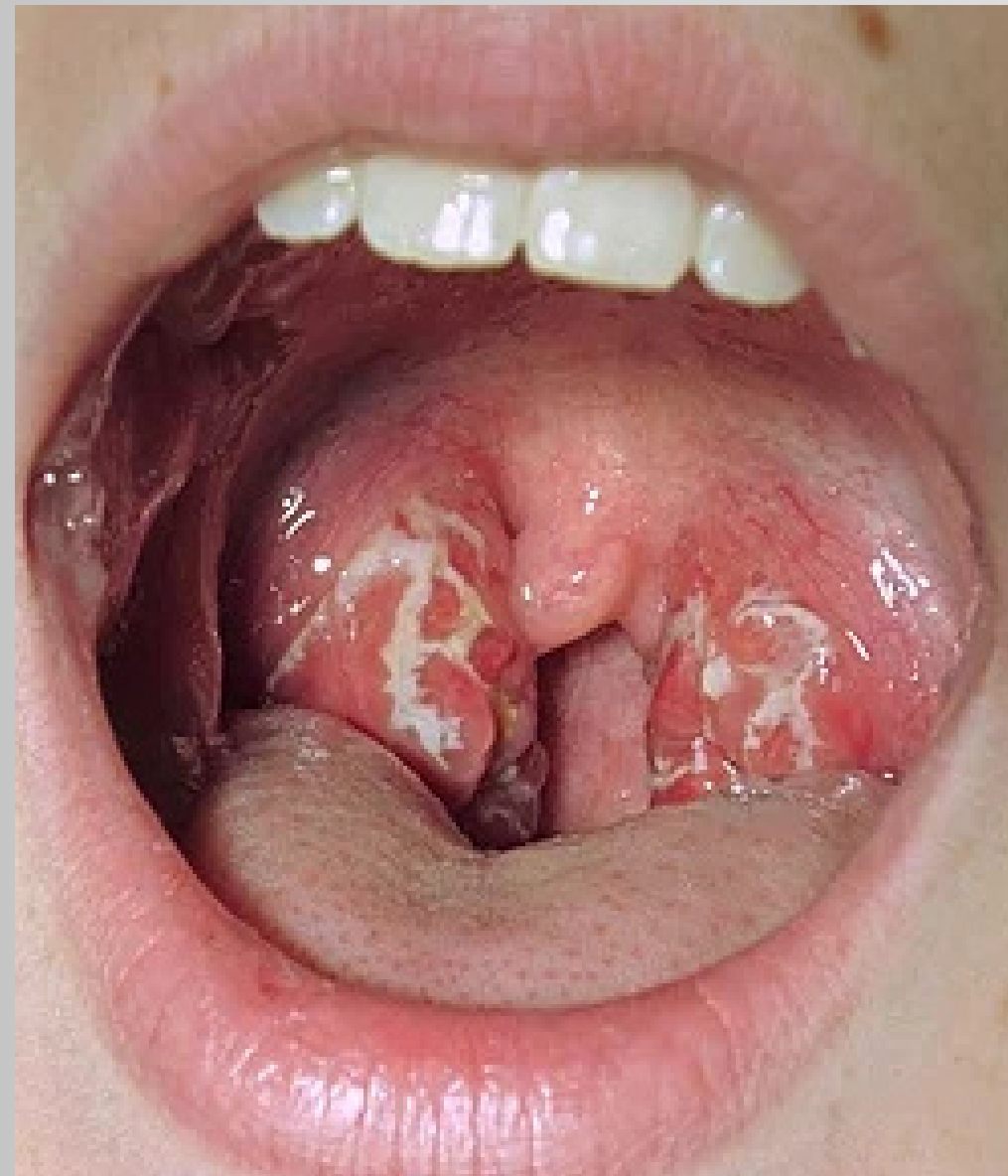
Not safe

Breast feeding



Flurbiprofen

Tonsillitis



Definition

1

Inflammation of the palatine tonsils due to infection (viral or bacterial)

Age

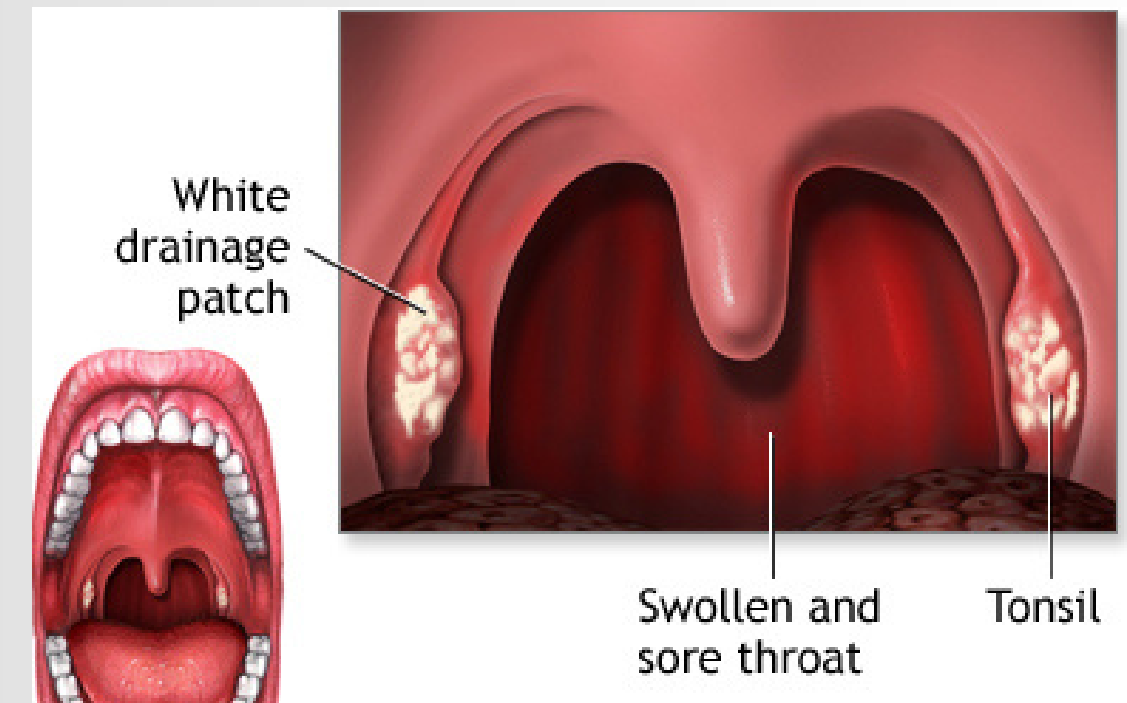
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Children (5–15 years)

features

3

sore throat, fever, and tonsillar enlargement $\pm$ exudate



Cause

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graph TD; Cause[Cause] --> Viral[Viral (70-80%)]; Cause --> Bacterial[Bacterial 20-30%];
```

Viral (70– 80%)

Rhinovirus, adenovirus, coronavirus, influenza

Bacterial 20–30%

Group **A** β -hemolytic streptococcus (**S. pyogenes**) – most important; also Groups C & G strep, **Staphylococcus aureus**, **Haemophilus influenzae** Key pathogen for complications

Pathology

Pathogenesis Infection → crypt inflammation → erythema, edema, exudates

Immune response

Local + systemic inflammatory reaction → fever, lymphadenopathy

	Viral	Bacterial
ONSET	More gradual	Sudden
Throat pain	Mild to moderate	severe
fever	low	high>38
Cough, coryza, rhinorrhea	Common ✓	absent ✗
Tonsils	Red, swollen, little/no exudate ✗	Enlarged, purulent exudates ✓
Lymph nodes	Mild or absent ✗	Tender anterior cervical lymphadenopathy ✓

Viral

Bacterial

Duration

5–7 days, self-limiting

**7–10 days, needs antibiotics if
strep confirmed**

Systemic

**Malaise, fatigue,
conjunctivitis**

Headache, abdominal pain (children)

Complications

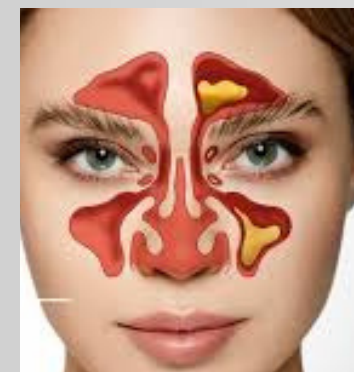
Suppurative (local)

1 severe sore throat

2 Retropharyngeal abscess Deep neck space infection, airway compromise

3 Otitis media Spread via Eustachian tube

4 Sinusitis Due to obstruction of sinus drainage



Non-suppurative (systemic, post-streptococcal)

1 Acute Rheumatic Fever (**ARF**)

2 Scarlet fever strawberry tongue, **sandpaper rash**

3 Post-streptococcal Glomerulonephritis (**PSGN**)

Immune complex deposition in kidneys
Hematuria, edema, hypertension, proteinuria

Management

1 NonPharmacological

hydration, warm saline gargles • Soft diet,
Avoid irritants (smoke, spicy food)



2 Pharmacological

1

Analgesics/antipyretics

Paracetamol, ibuprofen **First-line**

2

Topical anesthetics

Benzydamine spray, lidocaine/benzocaine
lozenges Local pain relief

3

NSAID lozenges

Flurbiprofen Some efficacy

4

Antibiotics

3 Tonsillectomy Indications

Obstructive sleep apnea from tonsillar hypertrophy • Recurrent peritonsillar abscess • Suspicion of malignancy

≥7 episodes in 1 year • ≥5 episodes/year for 2 years • ≥3 episodes/year for 3 years

Antibiotics

First-line

✓ **Penicillin V** (phenoxymethylpenicillin) 10-day course
Children: 12.5–25 mg/kg 2–4 times daily; **Adults:** 500 mg QDS



✗ **Amoxicillin 500 mg TDS 10 days**
Avoid if EBV Glandular fever (EBV) fever, malaise, rash, splenomegaly suspected (rash)

Alternatives (Penicillin Allergy)

✓ **Clarithromycin 250–500 mg BD 5 days** Better tolerated than erythromycin

✓ **Azithromycin 500 mg OD 3 days** Short course,

✗ **Erythromycin 250–500 mg QDS 7–10 days** GI side effects common

